



**Social Health
Authority**

ACCOUNTING POLICY PAPER INITIAL AND SUBSEQUENT MEASUREMENTS

SOCIAL HEALTH AUTHORITY

22nd May 2026

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IFRS 17 Insurance Contracts establishes principles for the recognition, measurement, presentation and disclosure of insurance contracts within the scope of the Standard. The objective of IFRS 17 is to ensure that an insurer provides relevant information that faithfully represents those contracts. This information gives a basis for users of financial statements to assess the effect that insurance contracts have on the insurer's financial position, financial performance and cash flows.

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OUR REF

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DOCUMENT PURPOSE

Submission to the Management of Social Health Authority

DOCUMENT OWNERSHIP

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ABBREVIATIONS & DEFINITIONS

NAME	DEFINITION
CSM	Contractual Service Margin
FCF	Fulfillment Cash Flows
GMM	General Measurement Model
IBN(E)R	Incurred But Not (Enough) Reported
IFRS	International Financial Reporting Standards
KAFS	Kenbright Advisory and Financial Services Limited
LIC	Liability for Incurred Claims
LRC	Liability for Remaining Coverage
PAA	Premium Allocation Approach
SHA	Social Health Authority
SHIF	Social Health Insurance Fund

1 INTRODUCTION

Background

- 1.1 The international Accounting Standards Board ('IASB') introduced IFRS 17 in May 2017 to supersede IFRS 4 which was an interim standard. IFRS 17 insurance contracts establishes the principles for the recognition, measurement, presentation, and disclosure of insurance contracts.
- 1.2 The objective of IFRS 17 is to ensure that the entity provides relevant information that faithfully represent contracts that fall within the scope of the Standard.
- 1.3 The Social Health Authority ('SHA') is a statutory agency established under Section 4 of the Social Health Insurance Act, which was enacted on 22nd November 2023 to succeed the National Hospital Insurance Fund (NHIF) as Kenya's primary health coverage provider. NHIF ceased its operations on **30th June 2024**, with its final admissions date being 30th September 2024. The transition period, spanning from November 2023 to mid-2024, involved the legal transfer of assets, liabilities and functions. SHA officially commenced its operations on **1st October 2024**.
- 1.4 SHA is mandated to pool contributions from individuals, employers, and the government in order to purchase healthcare services from empaneled and contracted healthcare providers and pay for the provision of quality healthcare services to members and their beneficiaries.
- 1.5 SHA manages three funds:
 - a) Primary Healthcare Fund;
 - b) Social Health Insurance Fund (SHIF);
 - c) Emergency, Chronic, and Critical Illness Fund (ECCF); and
 - d) Public Officers Medical Scheme Fund (POMSF)

Purpose

- 1.6 This policy paper outlines the application of IFRS 17 to the initial and subsequent measurement of insurance contracts issued by the Social Health Authority.
- 1.7 With respect to this, the policy paper expounds on the approaches laid down for the initial and subsequent measurement of insurance contracts in accordance with IFRS 17, provides an overview of its main elements and discusses the relevant adjustments.

Objective

- 1.8 The objective of this policy paper is to define the principles for initial and subsequent measurement of SHA's insurance contracts in accordance with IFRS 17.
- 1.9 It also aims to strengthen the financial management by ensuring clear, consistent, and compliant measurement practices across all funds issued by SHA.

Scope

- 1.10 This policy paper applies to all insurance contracts administered by SHA and outlines the application of initial and subsequent measurement principles as required under IFRS 17.

- 1.11 It provides guidance on the recognition, classification, and measurement of the Liability for Remaining Coverage (LRC) and the Liability for Incurred Claims (LIC), using either the Premium Allocation Approach (PAA) or, where applicable, the General Measurement Model (GMM).

2 MEASUREMENT AT INITIAL RECOGNITION

2.1 Under IFRS 17, insurance contracts paragraph 25 states that:

An entity shall recognize a group of insurance contracts it issues from the earliest of the following:

- a) the beginning of the coverage period of the group of contracts;
- b) the date on which the first payment from a policyholder in the group becomes due; and
- c) for a group of onerous contracts, when the group becomes onerous.

2.2 SHA applies this recognition guidance across its three health insurance schemes, adapting it to the operational timelines of enrolment, coverage activation, and contract servicing.

Assessment at Initial measurement

2.3 Upon recognition, SHA will undertake a structured assessment of the relevant financial and actuarial factors that will impact the valuation of the insurance contract.

2.4 This will involve determining the carrying amount for the Liability for Remaining Coverage (LRC), depending on the nature and duration of the contract boundary.

a) Contracts with a boundary of twelve (12) months or less.

2.5 For contracts with a duration of twelve (12) months or less, the PAA may be applied. In such cases, the LRC will be calculated using a simplified approach:

- a) **Premium received less Acquisition of Cost cash flows-** The LRC will be determined by subtracting directly attributable acquisition cash flows from the contributions received. Since every Kenyan household is mandated by law to contribute to the Fund, there are no insurance acquisition costs incurred by SHA, hence, the LRC will equal the contributions received.

2.6 All contracts issued by SHA fall under contracts with a boundary of twelve (12) months or less.

b) Contracts with a boundary exceeding twelve (12) months.

2.7 For any contracts, where the contract boundary may exceed twelve (12) months, the LRC shall be measured using the GMM which includes the Estimates of Future Cash Flows (FCFs), Adjustment for the Time Value of Money and Financial Risks, Risk Adjustment for Non-Financial Risks and Contractual Service Margin (CSM).

2.8 For coverage periods exceeding twelve (12) months, the PAA may still be applied, provided it can be reasonably expected that the resulting measurements would not differ materially from those produced under the GMM

2.9 However, given the current short-term nature of the contracts issued by the Social Health Authority (SHA) the simplified Premium Allocation Approach (PAA), as described above, shall be used to measure the LRC. In case any contracts exceeding twelve (12) months are to be issued in future, SHA will conduct the PAA eligibility test to determine whether PAA or GMM will be applicable.

3 SUBSEQUENT MEASUREMENT OF INSURANCE CONTRACT GROUPS

- 3.1 The subsequent measurement of insurance contracts will be essential for SHA as it will support the preparation of timely and accurate financial reports and ensure ongoing compliance with IFRS 17.
- 3.2 This process will involve the periodic reassessment of insurance liabilities, including the Liability for Remaining Coverage (LRC) and the Liability for Incurred Claims (LIC), in accordance with IFRS 17, paragraph 55(b) which states:

at the end of each subsequent reporting period, the carrying amount of the liability is the carrying amount at the start of the reporting period:

- i. plus the premiums received in the period;
- ii. minus insurance acquisition cash flows; unless the entity chooses to recognise the payments as an expense applying paragraph 59(a);
- iii. plus any amounts relating to the amortisation of insurance acquisition cash flows recognised as an expense in the reporting period; unless the entity chooses to recognise insurance acquisition cash flows as an expense applying paragraph 59(a);
- iv. plus any adjustment to a financing component, applying paragraph 56;
- v. minus the amount recognised as insurance revenue for services provided in that period (see paragraph B126); and
- vi. minus any investment component paid or transferred to the liability for incurred claims

Components of subsequent measurement

- 3.3 SHA will measure its insurance obligations after initial recognition using two key components:
- a) **Liability for Remaining Coverage (LRC)**-The LRC will represent SHA's obligation to provide insurance coverage for the remaining period of the insurance contracts across its health schemes.
 - b) **Liability for Incurred Claims (LIC)**-The LIC will reflect SHA's obligation to settle claims that have been incurred but not yet paid, including both reported and unreported claims.

Adjustments to the Liability for Remaining Coverage (LRC)

a) Contracts with boundaries of twelve (12) months or less

- 3.4 SHA will apply a simplified adjustment methodology, as follows:



- 3.5 Since every Kenyan household is mandated by law to contribute to the Fund, the LRC will not take into account insurance acquisition costs.

b) Contracts with boundaries exceeding twelve (12) months

- 3.6 Given that SHA issues short-term insurance contracts, the Premium Allocation Approach (PAA) will be the applicable measurement model.

Adjustments to the Liability for Incurred Claims (LIC)

- 3.7 SHA will adjust the LIC for the time value of money and financial risks, especially when claims are expected to be settled more than one year after the date of occurrence.
- 3.8 These adjustments will ensure that the Liability for Incurred Claims (LIC) continues to accurately reflect SHA's outstanding claims obligations.

Components of LIC Reserves

- 3.9 The LIC reserves will be established in accordance with internal actuarial guidelines, developed by the appointed Fund Actuary. These reserves will comprise:
- a) **Outstanding Claims Reserves (OCR):** Reserves for claims reported but not yet settled.
 - b) **Incurred But Not (Enough) Reported (IBN(E)R):** Reserves for claims that have been incurred but not yet reported or not sufficiently reported.
 - c) **Risk Adjustment (RA):** The risk adjustment reflects the uncertainty SHA faces in delivering health services, such as unpredictable claims. SHA will set this aside to enhance service delivery and ensure sustainability of the Fund.
 - d) **Unallocated Loss Adjustment Expenses (ULAE):** Reserves for the estimated indirect expenses related to claims handling and settlement that are not directly linked to individual claims.

4 ASSESSMENT OF ONEROUS INSURANCE CONTRACTS

4.1 Onerous contracts, in the context of the PAA, are subject of paragraphs 18 and 57, which state:

- 18 For contracts issued to which an entity applies the premium allocation approach, the entity shall assume no contracts in the portfolio are onerous at initial recognition, unless facts and circumstances indicate otherwise. An entity shall assess whether contracts that are not onerous at initial recognition have no significant possibility of becoming onerous subsequently by assessing the likelihood of changes in applicable facts and circumstances.
- 57 If at any time during the coverage period, facts and circumstances indicate that a group of insurance contracts is onerous, an entity shall calculate the difference between:
- a) the carrying amount of the liability for remaining coverage determined; and
 - b) the fulfilment cash flows that relate to remaining coverage of the group.

4.2 SHA will be required to assess whether groups of insurance contracts are onerous to ensure accurate financial reporting and effective risk management.

4.3 This assessment will involve evaluating expected cash flows and potential losses at both initial and subsequent measurement stages.

4.4 The methodology will differ based on the duration of the contract boundary.

Contracts with boundaries of twelve (12) months or less

At Initial Recognition

4.5 SHA will consider a contract to be onerous if the expected combined ratio for the portfolio exceeds 100%. The combined ratio is calculated as:

$$\text{Combined ratio} = \frac{\text{Insurance service expenses}}{\text{Insurance revenue}} * 100$$

4.6 The insurance service expenses entail:

- a) Incurred claims and other directly attributable expenses
- b) Changes that relate to past service- adjustments to the LIC
- c) Losses on onerous contracts
- d) Insurance acquisition cash flows amortization- Since SHA membership is mandatory for every household, this will not be applicable

4.7 The assessment will also include consideration of the combined ratios of previous cohorts, providing a historical perspective on performance.

At Subsequent measurement

4.8 SHA will reassess the status of contracts at each reporting period to determine whether they remain onerous.

4.9 This will be based on updated data and prevailing conditions, including:

- a) **Current expectation of Combined Ratios**-Ongoing evaluation of combined ratios to determine if they exceed 100%.
- b) **Internal factors**-Unfavourable changes in the loss ratio, indicating higher-than-expected losses.
- c) **External factors**-Industry-wide loss ratios provided by SHA, offering insights into broader market conditions.

Contracts with boundaries exceeding twelve (12) months

4.10 No contracts issued by SHA have contract boundaries exceeding twelve (12) months

5 REFERENCE

Refer to the IFRS 17 Standard available online at the address below

<https://www.ifrs.org/content/dam/ifrs/publications/pdf-standards/english/2022/issued/part-a/ifrs-17-insurance-contracts.pdf?bypass=on>

This policy has been adopted and signed by the Board of Management of the Social Health Authority on this _____ day of _____ 2025.

Chief Executive Officer: _____

Signature: _____

Appointed Actuary: Ezekiel Macharia

Signature: _____

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